

LIPISTOROL CAPSULE 300mg

VLLIPxx-0-FEO

DESCRIPTION

Maroon opaque and white opaque capsule with "HD" printed on one end and "GMB300" on the other end of the capsule.

COMPOSITION

Gemfibrozil 300 mg / capsule

ACTION AND PHARMACOLOGY

Pharmacodynamic

Gemfibrozil is a lipid regulating agent which regulates lipid fractions.

Gemfibrozil's mechanism of action has not been definitively established. In man, gemfibrozil stimulates the peripheral lipolysis of triglyceride rich lipoproteins such as VLDL and chylomicrons (by stimulation of LPL). Gemfibrozil also inhibits synthesis of VLDL in the liver. Gemfibrozil increases the HDL2 and HDL3 subfractions as well as apolipoprotein A-I and A-II.

Pharmacokinetic

Absorption

Gemfibrozil is well absorbed from the gastro-intestinal tract after oral administration with a bioavailability close to 100%. As the presence of food alters the bioavailability slightly gemfibrozil should be taken 30 minutes before a meal. Peak plasma levels occur in one to two hours. After administration of 600 mg twice daily a Cmax in the range 15 to 25 mg/ml is obtained.

Distribution

Volume of distribution at steady state is 9-13 l. The plasma protein binding of gemfibrozil and its main metabolite are at least 97%.

Biotransformation

Gemfibrozil undergoes oxidation of a ring methyl group to form successively a hydroxymethyl and a carboxyl metabolite (the main metabolite). This metabolite has a low activity compared to the mother compound gemfibrozil and an elimination half-life of approximately 20 hours.

The enzymes involved in the metabolism of gemfibrozil are not known. The interaction profile of gemfibrozil is complex (see sections 4.3, 4.4 and 4.5). In vitro and in vivo studies have shown that gemfibrozil inhibits CYP2C8, CYP2C9, CYP2C19, CYP1A2, UGT1A1 and UGT3A3.

Elimination

Gemfibrozil is eliminated mainly by metabolism. Approximately 70% of the administered human dose is excreted in the urine, mainly as conjugates of gemfibrozil and its metabolites. Less than 6% of the dose is excreted unchanged in the urine. Six percent of the dose is found in faeces. The total clearance of gemfibrozil is in the range 100 to 160 ml/min, and the elimination half-life is in the range 1.3 to 1.5 hours. The pharmacokinetics is linear within the therapeutic dose range

INDICATIONS

Gemfibrozil is indicated as an adjunct to diet and other therapeutic measures for:

- Treatment of adult patients with very high serum triglyceride levels, Fredrickson classification Type III, IV and V hyperlipidemias.
- Treatment of patients with hypercholesterolemia, type IIa and IIb mixed dyslipidemias, to regulate lipid (reduce serum triglycerides and LDL cholesterol levels and increase HDL cholesterol).

CONTRAINDICATIONS

- Hypersensitivity to the active substance.
- Hepatic impairment.
- Severe renal impairment.
- History of/or pre-existing gall bladder or biliary tract disease, including gallstones
- Concomitant use of repaglinide or simvastatin.
- Patients with previous history of photoallergy or phototoxic reaction during treatment with fibrates.

WARNING AND PRECAUTIONS

There have been reports of myositis, myopathy and markedly elevated creatine phosphokinase associated with gemfibrozil. Rhabdomyolysis has also been reported rarely. Muscle damage must be considered in any patient presenting with diffuse myalgia, muscle tenderness and/or marked increase in muscle CPK levels (>5x ULN); under these conditions treatment must be discontinued.

The concomitant administration of gemfibrozil with simvastatin is contraindicated. There have been reports of severe myositis with markedly elevated creatine kinase and myoglobinuria (rhabdomyolysis) when gemfibrozil and HMG CoA reductase inhibitors were used concomitantly. Pharmacokinetic interactions may also be present and dosage adjustments may be necessary. The benefit of further alterations in lipid levels by the combined use of gemfibrozil and HMG-CoA reductase inhibitors should be carefully weighed against the potential risks of such combinations and clinical monitoring is recommended. A creatine phosphokinase (CPK) level should be measured before starting such a combination in patients with pre-disposing factors for rhabdomyolysis.

In most subjects who have had an unsatisfactory lipid response to either drug alone, the possible benefits of combined therapy with HMG-CoA reductase inhibitors and gemfibrozil does not outweigh the risks of severe myopathy, rhabdomyolysis and acute renal failure.

Gemfibrozil may increase cholesterol excretion into the bile raising the potential for gallstone formation. If cholelithiasis is suspected, gallbladder studies are indicated. Gemfibrozil therapy should be discontinued if gallstones are found.

Periodic determinations of serum lipids are necessary during treatment with gemfibrozil. If the response is insufficient after 3 months of therapy at recommended doses treatment should be discontinued and alternative treatment methods considered.

Elevated levels of ALAT, ASAT, alkaline phosphatase, LDH, CK and bilirubin have been reported. These are usually reversible when gemfibrozil is discontinued. Therefore liver function tests should be performed periodically. Gemfibrozil therapy should be terminated if abnormalities persist.

Periodic blood count determinations are recommended during the first 12 months of gemfibrozil administration.

There is insufficient information on the effects on the ability to drive and use machines. In isolated cases dizziness and visual disturbances can occur which may negatively influence driving.

PREGNANCY AND LACTATION

No adequate data on use of Gemfibrozil in pregnant and lactating women. Gemfibrozil should not be used during pregnancy unless it is clearly necessary. Gemfibrozil should not be used when breast-feeding.

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DRUG INTERACTIONS

- The interaction profile of gemfibrozil is complex. Gemfibrozil is a potent inhibitor of CYP2C8 and strong inhibitor of CYP2C9, CYP 2C19, CYP1A2 and UGT1A1 and UGT3A3.
- The combination of gemfibrozil with repaglinide is contra-indicated. Concomitant administration has resulted in 8-fold increase in repaglinide plasma concentration probably by inhibition of the CYP2C8 enzyme, resulting in hypoglycaemic reactions.
- The combination of gemfibrozil with rosiglitazone should be approached with caution. Co-administration with rosiglitazone has resulted in 2.3-fold increase in rosiglitazone systemic exposure, probably by inhibition of the CYP2C8 isozyme
- The concomitant administration of gemfibrozil with simvastatin is contraindicated. An increased risk of muscle related adverse events, including rhabdomyolysis, has been reported when fibrates are co-administered with statins. Gemfibrozil has also been reported to influence the pharmacokinetics of simvastatin, lovastatin, pravastatin and rosuvastatin. Gemfibrozil caused an increase in AUC of simvastatin, pravastatin and rosuvastatin.
- Gemfibrozil may potentiate the effects of coumarin type vitamin K antagonist anticoagulants such as warfarin, acenocoumarol, or phenprocoumon. The concomitant administration of gemfibrozil with these anticoagulants necessitates careful monitoring of prothrombin time (INR).
- Concomitant administration of gemfibrozil with bexarotene is not recommended.
- Reduced bioavailability of gemfibrozil may result when given simultaneously with resin-granule drugs such as colestipol. Administration of the products two hours or more apart is recommended.
- Risk of myopathy and rhabdomyolysis may be increased with concomitant administration of colchicine and gemfibrozil. This risk may be increased in the elderly and in patients with hepatic or renal dysfunction. Clinical and biological monitoring are recommended, especially at the start of combined treatment.
- Gemfibrozil is highly bound to plasma proteins and there is potential for displacement interactions with other drugs.

MAIN SIDE / ADVERSE EFFECTS

- Blood and lymphatic system disorders**
Rare: Bone marrow failure, severe anaemia, thrombocytopenia, leukopenia, eosinophilia
- Psychiatric disorders**
Rare: Depression, decreased libido
- Nervous system disorders**
Common: Vertigo, headache
Rare: Neuropathy peripheral, paraesthesia, dizziness, somnolence
- Eye disorders**
Rare: Vision blurred
- Cardiac disorders**
Uncommon: Atrial fibrillation
- Respiratory, thoracic and mediastinal disorders**
Rare: Laryngeal oedema

Gastrointestinal disorders

Very common: Dyspepsia

Common: Diarrhoea, vomiting, nausea, abdominal pain constipation, flatulence

Rare: Pancreatitis, appendicitis

Hepatobiliary disorders

Rare: Jaundice cholestatic, hepatitis, cholelithiasis, cholecystitis, hepatic function abnormal

Skin and subcutaneous tissue disorders

Common: Eczema, rash

Rare: Angioedema, dermatitis exfoliative, urticaria, dermatitis, alopecia, photosensitivity reaction, pruritus

Musculoskeletal and connective tissue disorders

Rare: Rhabdomyolysis, myopathy, myositis, muscular weakness, synovitis, myalgia, arthralgia, pain in extremity

Reproductive system and breast disorder

Rare: Erectile dysfunction

General disorders and administration site conditions

Common: Fatigue

Investigations

Rare: Haemoglobin decreased, haematocrit decreased, white blood cell count decreased, blood creatine phosphokinase increased.

OVERDOSE

Overdose has been reported. Symptoms reported with overdosage were abdominal cramps, abnormal LFT's, diarrhea, increased CPK, joint and muscle pain, nausea and vomiting. The patients fully recovered. Symptomatic supportive measures should be taken if overdose occurs.

DOSAGE AND ADMINISTRATION

Adult:

Oral, 1.2 g a day in two divided doses, thirty minutes before morning and evening meals.

Paediatric:

Dosage has not been established.

The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:

Store below 25°C. Protect from moisture.

Presentation/Packing

Bottle of 100's

Product Registration Holder: HOVID Bhd.

121, Jalan Tunku Abdul Rahman,
30010 Ipoh, Malaysia.

Manufactured by: HOVID Bhd.

Lot 56442, 7 1/2 miles, Jalan Ipoh/Chemor,
31200 Chemor, Malaysia.

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